

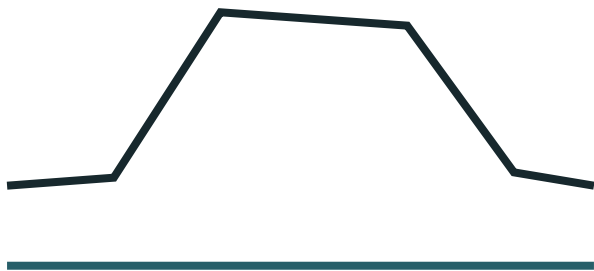
# Retinal detachment operative selection

Part 2 | Identify the plane and the force

An elevated retina does not identify the operative mechanism.

## RRD

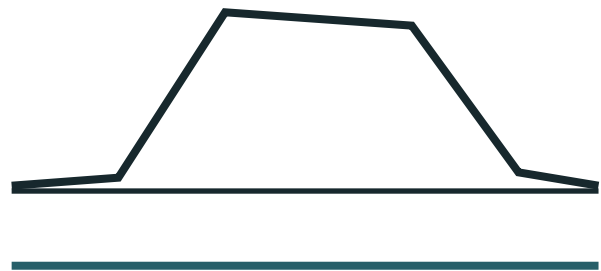
Subretinal separation



RPE

## Retinoschisis

Split within retina



RPE

Original section: subretinal separation versus an intraretinal split.

Map breaks, traction, macular involvement and proliferative change before naming an operation. A rhegmatogenous detachment needs break closure; tractional disease needs assessment of the force maintaining elevation. Stable degenerative schisis is a different process. In an oral case, state the plane first, then explain why the proposed intervention changes that mechanism.

# A buckle is an anatomical selection

A young phakic eye without PVD can favour external support.

| Finding                      | Implication                                   |
|------------------------------|-----------------------------------------------|
| Anterior identifiable breaks | External support can close the relevant break |
| Clear phakic lens            | Consider preserving the lens and vitreous     |
| Advanced PVR                 | May require internal traction relief          |
| Encircling element           | Counsel axial refractive and motility effects |

Choice is not determined by age alone. Relate lens state, complete break map, vitreous attachment, scleral integrity and surgeon experience to the plan. Explain potential myopic shift and diplopia as anatomical consequences. An operation can attach the retina successfully while introducing a new optical or binocular problem that matters to the patient.

# Pneumatic retinopexy needs a suitable break map

Selection and positioning are integral to the procedure.

| Finding                                 | Implication                         |
|-----------------------------------------|-------------------------------------|
| Restricted superior causative breaks    | Potentially suitable gas coverage   |
| Inferior causative break                | Bubble coverage may be inadequate   |
| Clear media and cooperative positioning | Needed for assessment and aftercare |
| Missed or new tear                      | Can cause primary failure           |

Describe where the detached-retina breaks lie rather than saying that no inferior break can ever coexist. Selected trial eyes may have breaks in attached inferior retina. The decision combines break geometry, media clarity, PVR and capacity for close follow-up. A convenient outpatient procedure is not a substitute for a complete peripheral examination.

# Compare anatomical and patient-centred outcomes

Single-operation success and final visual function are different endpoints.

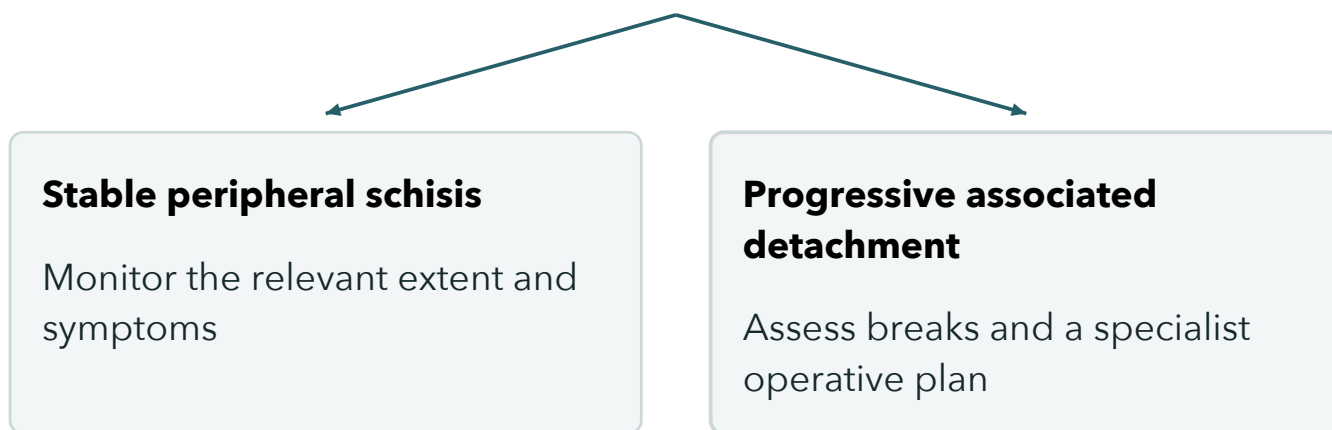
| Finding             | Implication                                        |
|---------------------|----------------------------------------------------|
| PIVOT population    | Selected detachments suitable for pneumatic repair |
| Comparison          | Pneumatic retinopexy versus vitrectomy             |
| Patient outcome     | Visual function can favour pneumatic repair        |
| Failure counselling | Discuss additional procedures and final attachment |

A trial comparison is useful only after checking its entry criteria. Separate primary attachment, final attachment, acuity, distortion and treatment burden. Ask the patient which consequences matter most, then discuss the trade-off without promising that a less invasive operation always has better results for a different break pattern.

# Schisis: do not operate on the label alone

Progression and a true detachment change the decision.

## What is happening beyond the split?



An inner-layer defect is not the same as a break through both retinal layers. Document the posterior edge, outer-layer involvement and any progressive subretinal separation. The oral challenge is to identify which new anatomical finding would change observation to intervention, rather than treating an incidental stable split as an ordinary rhegmatogenous detachment.